

D349

Psychiatric Mental Health Nurse Practitioner Clinical Internship II

Armored Drill

8 concepts · 12 items · 4 transfer items

What this is. Every concept in this course's public description, re-asked with more disguise than a practice test uses. Each stem carries facts that do not decide it. Every wrong answer is a named trap.

Source. Scope from the course's public description in the WGU Institutional Catalog (September 2026); items written from the field's standard body of knowledge. No WGU assessment item, course material or proprietary content is used or reproduced. Not affiliated with or endorsed by Western Governors University.

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How to run it

Paper first. Pencil. One pass, no notes, a timer if you want the pressure.

1. **Before you read the options**, fill the strip under the item: the target word (what is actually being asked), and the one fact in the stem that decides it.
2. **Principle before arithmetic**. Name the rule, then compute.
3. **Camouflage**. Most of every stem does not matter. Ask: which single fact decides this?
4. **Inverse pairs are the bait**. If two options are opposites, one of them is the trap built for you.
5. **Mark sure, unsure or guess** honestly. Sure-and-wrong is the most valuable miss on the page — it is a wrong mental model.
6. **Absolutes** (always, never, guarantee) are usually wrong. And recognizing an answer is not the same as owning it.
7. Items with a **heavy black bar** on the left are **transfer items**: the same principle, reversed or solved for a different variable.
8. Stop at the STOP page. Then grade, fill the triage grid, and tally the traps you fell for. The biggest tally is what you drill next.

The strip, filled in, looks like this:

Target word _____	The one fact that decides _____	Answer _____	[] sure [] unsure [] guess
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Items

1.

A 33-year-old nurse who works night shifts completes a GAD-7 at her follow-up visit. Her item scores are 3, 2, 2, 1, 3, 2, 2. She mentions she recently adopted a rescue dog. What are her total score and severity?

- A. 15, severe
- B. 15, moderate
- C. 2.1, minimal
- D. 13, moderate

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

2. [TRANSFER — reversed stem / different variable]

A patient's baseline GAD-7 is 16. The treatment target is a response of at least a 50% reduction. What score or lower meets the target?

- A. 10
- B. 8
- C. 4
- D. 24

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

3.

A patient started escitalopram 5 mg three weeks ago for major depression and reports slight improvement and no side effects. She is a paralegal who recently started yoga. What is the best next step?

- A. Switch to a different antidepressant now.
- B. Increase to a standard therapeutic dose and continue for several more weeks before judging response.
- C. Credit yoga and stop the medication.
- D. Add an antipsychotic augmenting agent.

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

4. [TRANSFER — reversed stem / different variable]

After 8 weeks at the maximum tolerated dose of sertraline, a patient's depression scores have improved less than 25%. What does current practice support?

- A.** Continue unchanged for another 6 months.
- B.** Reduce the dose.
- C.** Switch to another antidepressant or augment, and consider adding psychotherapy.
- D.** Stop all treatment, since medication does not work for this patient.

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

5.

A 27-year-old with schizophrenia who plays guitar has continued hallucinations after 8-week trials of risperidone and then olanzapine at adequate doses with good adherence. His mother drives him to appointments. What is the recommended next step?

- A.** Stop antipsychotics because none will work.
- B.** Start clozapine without blood monitoring.
- C.** Start clozapine with required ANC monitoring.
- D.** Try a third standard antipsychotic.

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

6.

Three days after abruptly stopping paroxetine, a patient reports dizziness, nausea, irritability, and electric-shock sensations. He recently returned from a work conference in Chicago. What is most likely?

- A.** Serotonin syndrome.
- B.** Relapse of depression.
- C.** Antidepressant discontinuation syndrome; restart and taper slowly.
- D.** A viral illness picked up while traveling.

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

7. [TRANSFER — reversed stem / different variable]

Which antidepressant is least likely to cause discontinuation symptoms if doses are missed?

- A.** Venlafaxine
- B.** Fluoxetine
- C.** All antidepressants carry equal risk
- D.** Paroxetine

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

8.

In a weekly depression group that meets in a church basement, a new member says, “I thought I was the only one who felt this way. Hearing you all makes me feel less alone.” Eight members attend regularly. Which therapeutic factor is she describing?

- A. Altruism
- B. Spiritual support from the setting
- C. Instillation of hope
- D. Universality

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

9.

A 24-year-old graphic designer with newly diagnosed bipolar I disorder is sexually active and does not use contraception. She has a history of migraines. Her roommate takes valproate. Which statement guides the mood stabilizer choice?

- A. Valproate is fine because her roommate tolerates it.
- B. Valproate is preferred because it also helps migraines.
- C. No mood stabilizer can ever be used in women of childbearing age.
- D. Avoid valproate if possible because of high risk of neural tube defects; consider alternatives and discuss contraception.

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

10.

A rural primary care clinic with two family physicians adds a behavioral health care manager who tracks patients in a registry, and a psychiatric NP who reviews the caseload weekly and advises on treatment. The clinic also added evening hours. What model is this?

- A. Collaborative care model
- B. Traditional referral to specialty psychiatry
- C. Extended hours access model
- D. Assertive community treatment

Target word	The one fact that decides	Answer	[] sure [] unsure [] guess
_____	_____	___	

11.

A 60-year-old retired teacher has taken clonazepam 2 mg daily for five years and wants to stop. She volunteers at the library and walks every morning. What approach is safest?

- A.** A gradual taper, reducing about 10-25% of the current dose every 2-4 weeks, with adjustments for symptoms.
- B.** Stop today, since she wants to quit.
- C.** Halve the dose each week until off.
- D.** Rely on her healthy habits to prevent withdrawal.

Target word _____	The one fact that decides _____	Answer ____	[] sure [] unsure [] guess
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12. [TRANSFER — reversed stem / different variable]

A patient takes lorazepam 4 mg daily. The plan is to reduce by 25% of the current dose at each step. What is the daily dose after two steps?

- A.** 1 mg
- B.** 2 mg
- C.** 3 mg
- D.** 2.25 mg

Target word _____	The one fact that decides _____	Answer ____	[] sure [] unsure [] guess
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STOP

Answers and dissection follow. Finish every item and every strip first.

Key and dissection

Struck-through text is the camouflage: facts that do not decide the item.

1. Answer A Concept 1

A 33-year-old nurse ~~who works night shifts~~ completes a GAD-7 at her follow-up visit. Her item scores are 3, 2, 2, 1, 3, 2, 2. ~~She mentions she recently adopted a rescue dog.~~ What are her total score and severity?

Target word: total and band **Decides:** Sum = 15.

A	Correct	$3+2+2+1+3+2+2 = 15$; 15-21 is severe.
B	Adjacent category	Moderate is 10-14.
C	Inverted operation	Averages instead of summing.
D	Dropped element	Leaves out the last item ($3+2+2+1+3+2 = 13$).

2. Answer B Concept 1 • transfer

A patient's baseline GAD-7 is 16. The treatment target is a response of at least a 50% reduction. What score or lower meets the target?

Target word: solve for target score **Decides:** 16×0.5 .

A	Wrong stem number	The moderate cutoff, not 50% of baseline.
B	Correct	A 50% reduction from 16 is 8.
C	Confusion set	Under 5 is the remission threshold, not the response target.
D	Inverted operation	Adds 50% instead of subtracting it.

3. Answer B Concept 2

A patient started escitalopram 5 mg three weeks ago for major depression and reports slight improvement and no side effects. ~~She is a paralegal who recently started yoga.~~ What is the best next step?

Target word: adequate trial **Decides:** Low dose, short duration, tolerated.

A	Inverted operation	Switching before an adequate trial wastes a possibly effective drug.
B	Correct	The trial is not yet adequate in dose or duration.
C	Camouflage	The new activity does not decide medication management.
D	Adjacent category	Augmentation follows an adequate trial with partial response.

4. Answer C Concept 2 • transfer

After 8 weeks at the maximum tolerated dose of sertraline, a patient's depression scores have improved less than 25%. What does current practice support?

Target word: after an adequate trial fails **Decides:** Adequate dose and duration completed.

A	Dropped element	Ignores that the trial was adequate.
B	Inverted operation	Lowering the dose will not improve response.
C	Correct	Minimal response after an adequate trial calls for a change.
D	Absolute language	One failed trial does not rule out others.

5. Answer C Concept 3

A 27-year-old with schizophrenia ~~who plays guitar~~ has continued hallucinations after 8-week trials of risperidone and then olanzapine at adequate doses with good adherence. ~~His mother drives him to appointments.~~ What is the recommended next step?

Target word: treatment-resistant schizophrenia **Decides:** Two adequate antipsychotic trials failed.

A	Absolute language	Clozapine is often effective after others fail.
B	Dropped element	ANC monitoring is required for neutropenia risk.
C	Correct	Clozapine is indicated after two failed adequate trials.
D	Adjacent category	Delays the most effective option for resistant illness.

6. Answer C Concept 4

Three days after abruptly stopping paroxetine, a patient reports dizziness, nausea, irritability, and electric-shock sensations. ~~He recently returned from a work conference in Chicago.~~ What is most likely?

Target word: discontinuation vs. relapse **Decides:** Short half-life drug stopped abruptly, symptoms within days.

A	Inverted operation	That comes from too much serotonin, not stopping a drug.
B	Confusion set	Relapse develops over weeks and lacks shock sensations.
C	Correct	Classic timing and symptoms after stopping a short half-life drug.
D	Camouflage	Travel is incidental; the timing points to stopping the drug.

7. Answer B Concept 4 • transfer

Which antidepressant is least likely to cause discontinuation symptoms if doses are missed?

Target word: reverse: lowest risk **Decides:** Long half-life with active metabolite.

A	Confusion set	Short half-life; high risk.
B	Correct	Its long half-life self-tapers.
C	Absolute language	Risk varies with half-life.
D	Inverted operation	Short half-life; among the highest risk.

8. Answer D Concept 5

In a weekly depression group ~~that meets in a church basement~~, a new member says, "I thought I was the only one who felt this way. Hearing you all makes me feel less alone." ~~Eight members attend regularly.~~ Which therapeutic factor is she describing?

Target word: Yalom factor **Decides:** Realizing others share her experience.

A	Confusion set	Altruism is gaining from helping others.
B	Camouflage	The location is not a therapeutic factor.
C	Adjacent category	Hope comes from seeing others improve.
D	Correct	Recognizing that others share similar struggles.

9. Answer D Concept 6

A 24-year-old graphic designer with newly diagnosed bipolar I disorder is sexually active and does not use contraception. She has a history of migraines. ~~Her roommate takes valproate.~~ Which statement guides the mood stabilizer choice?

Target word: valproate risk **Decides:** Childbearing potential without contraception.

A	Camouflage	Another person's tolerance is irrelevant.
B	Dropped element	Ignores pregnancy risk.
C	Absolute language	Safer options exist with counseling.
D	Correct	Valproate is highly teratogenic.

10. Answer A Concept 7

A rural primary care clinic with two family physicians adds a behavioral health care manager who tracks patients in a registry, and a psychiatric NP who reviews the caseload weekly and advises on treatment. ~~The clinic also added evening hours.~~ What model is this?

Target word: name the model **Decides:** Care manager, registry, psychiatric consultant, caseload review.

A	Correct	These are its core elements.
B	Adjacent category	Referral moves care out of primary care without shared tracking.
C	Camouflage	Evening hours are not the defining feature.
D	Confusion set	ACT is a mobile team for severe mental illness.

11. Answer A Concept 8

A 60-year-old retired teacher has taken clonazepam 2 mg daily for five years and wants to stop. ~~She volunteers at the library and walks every morning.~~ What approach is safest?

Target word: benzodiazepine discontinuation **Decides:** Long-term daily use.

A	Correct	Slow tapering prevents withdrawal and seizures.
B	Inverted operation	Abrupt stop risks severe withdrawal and seizures.
C	Adjacent category	Too fast for five years of use.
D	Camouflage	Exercise does not prevent physiological withdrawal.

12. Answer D Concept 8 • transfer

A patient takes lorazepam 4 mg daily. The plan is to reduce by 25% of the current dose at each step. What is the daily dose after two steps?

Target word: dose after two percentage steps **Decides:** Each cut is 25% of the current dose.

A	Wrong stem number	Treats 25% of 4 mg (1 mg) as the resulting dose.
B	Inverted operation	Subtracts 25% of the original dose twice instead of the current dose.
C	Dropped element	Only one step.
D	Correct	$4 \times 0.75 = 3$; $3 \times 0.75 = 2.25$.

Triage grid

Write each item number in the box it belongs to.

	Right	Wrong
Sure	Owned.	Concept inversion — top priority. A wrong mental model; it survives rewording.
Unsure / guess	Recognition, not mastery. Re-drill with the transfer.	Not learned yet.

Trap tally

For every miss, find the letter you chose in the key and tally its trap. The biggest number is the pattern to drill.

Trap	What it does	In this drill	Your misses
Dropped element	Leaves out a required term or step.	5	
Wrong stem number	A figure copied from the prompt that is not the answer.	2	
Adjacent category	The right idea in the wrong bucket.	6	
Inverted operation	Direction flipped: add/subtract, cause/effect, higher/lower.	8	
Absolute language	Always, never, guarantee — where judgment applies.	4	
Confusion set	Every option is real; the test is exact role-matching.	5	
Camouflage	Answers a stem fact that was never the question.	6	

Anchor rows — the concept list

One row per concept: the rule to lock in, the trap that catches people, and the tell in the question that signals it.

#	Concept	Rule	Trap	Tell
1	Measurement-based care with the GAD-7	Sum seven items scored 0–3 (0–21): 5 mild, 10 moderate, 15 severe; track scores over time, with a 50% drop from baseline as a common response target.	Averaging items or reading the wrong band.	Item scores or a baseline and target score.
2	Adequate antidepressant trial	Judge an antidepressant only after an adequate dose for about 4–8 weeks; if response is minimal after that, switch or augment.	Switching after 2–3 weeks or at a sub-therapeutic dose.	Weeks on a drug and whether the dose was optimized.
3	Treatment-resistant schizophrenia	After two adequate trials of different antipsychotics fail, clozapine is indicated, with required ANC monitoring for neutropenia.	Trying a third or fourth standard antipsychotic, or skipping ANC monitoring.	Two failed antipsychotic trials.
4	Antidepressant discontinuation syndrome	Stopping short half-life agents (paroxetine, venlafaxine) abruptly can cause dizziness, flu-like symptoms, irritability, and electric-shock sensations; taper gradually; fluoxetine's long half-life makes it least likely to cause this.	Mistaking discontinuation symptoms for relapse or stopping abruptly.	New symptoms within days of stopping or missing doses.
5	Group therapeutic factors	Yalom's factors include universality (not alone), instillation of hope, altruism, interpersonal learning, and cohesiveness.	Confusing universality with altruism or cohesion.	A group member's statement about their experience in group.
6	Valproate and reproductive potential	Valproate carries high risk of neural tube defects and other harms in pregnancy; avoid it in people who could become pregnant unless no alternative, with counseling and effective contraception.	Choosing valproate by default for bipolar disorder in a young woman.	A mood stabilizer decision for a patient of childbearing potential.
7	Collaborative care model	In collaborative care, the primary care clinician, a behavioral care manager, and a psychiatric consultant use a registry and regular caseload reviews to treat to target.	Thinking it means a one-time referral to psychiatry.	Mental health care delivered inside primary care with a care manager.
8	Benzodiazepine tapering	Taper long-term benzodiazepines gradually (about 5–25% of the current dose every 2–4 weeks) to avoid withdrawal and seizures; never stop abruptly after long use.	Stopping all at once or cutting by a fixed amount of the original dose.	A patient on a benzodiazepine for months or years who wants to stop.

Course scope (public description): Through precepted clinical experiences, the learner will develop competencies needed to provide comprehensive and holistic mental health care to individuals, families, and communities across the lifespan. In the precepted clinical setting, the learner will combine competencies developed in preparatory advanced practice coursework to deliver consumer- centered mental health care. Therefore, the learner will conduct advanced mental health assessments and utilize the competencies of advanced pathophysiology, psychopharmacology, psychotherapy and health promotion for individuals and populations across the life span to build self-efficacy in individuals and groups, to determine correct diagnoses, and to establish mental health plans of care that include consumer and population preferences. All MSN Core Courses, NP Core courses, and PMHNP Specialty courses must be completed before taking this course.